Arolygiaeth Gofal Cymru
Care Inspectorate Wales

Inspection Report

52 Narberth Road

52 Narberth Road, Cardiff, CF5 5EY

02920 202 509

The inspection visit took place on 26/03/2026

Service Information:

Operated by:	Pilipala Care Ltd
Care Type:	Care Home Service Adults Without Nursing
Provision for:	Care home for adults - with personal care, Provision for learning disability, Provision for mental health
Registered places:	6
Main language(s):	English
Promotion of Welsh language and culture:	The provider makes an effort to promote the use of the Welsh language and culture or is working towards a bilingual service.

Ratings:

Well-being	Good
Care & Support	Good
Environment	Good
Leadership & Management	Good

Summary:

52 Narberth Road provides a welcoming, comfortable environment which meets the needs of the people who live there.
Wellbeing is rated ‘good’. People are treated with dignity and respect and are supported to make choices about their daily lives. Independence is encouraged, and we saw people being supported to maintain both their physical and emotional health.
Care and Support is rated ‘good’. The service delivers compassionate, person-centred care. Staff know people extremely well and demonstrate warmth, skill and attentiveness in their interactions. Communication is effectively supported, health needs are proactively monitored, and people told us they enjoy living at the home.
Environment is rated ‘good’. The home is warm, inviting and comfortable. Routine redecoration and furniture renewal is planned following some internal work on doorways. There are also plans to make layout changes to suit the needs of people.
Leadership and Management is rated ‘good’. The provider demonstrates effective oversight and promotes a positive organisational culture. Good outcomes are consistently achieved due to the provider’s commitment to developing and retaining a skilled and competent workforce.

Findings:

Well-being — Good

People are treated with dignity and respect and benefit from warm, respectful and nurturing relationships with staff who know them well. People are actively involved in shaping daily life. Monthly house meetings, supported with picture communication, enable individuals to contribute to decisions about menus, activities and household tasks. Staff were consistently seen encouraging and supporting people in daily routines. We found records do not demonstrate people are involved in identification of their outcomes and goals, though we are told this is the case. The provider is taking action to ensure this is captured. One resident told us, "*I love it here*," describing staff as "*excellent*."

People consistently told us they are happy living at the home, with comments such as "*I like living here*." Throughout the inspection, we observed relaxed, positive interactions between people and staff, characterised by humour, encouragement and meaningful conversation. Keyworkers (primary workers), in particular show a strong understanding of people's histories, preferences, talents and communication styles.

People generally experience a good range of day-to-day activity and stimulation. We saw individuals participating in preferred activities including jigsaws, music, television, dominoes, shopping outings and celebrating birthdays. Staff encourage participation in household tasks and support people to maintain routines that promote independence. Although one person reported they were not encouraged to help, their keyworker provided several examples demonstrating this is not the case. However, staffing pressures have recently impacted community participation for some people. Rotas, supervision records and documentation we considered confirmed periods where staff sickness or resident ill-health reduced opportunities for community outings. While staffing levels meet the minimum daytime requirement, the impact on meaningful activity and community access has affected well-being for some individuals.

Communication needs are understood and respected. One person has a detailed communication profile supported by flash cards and clear staff guidance. Accessible tools such as picture-based surveys and green/amber/red feedback symbols enable people to express their views and exercise choice.

People feel safe living at the home and know who they can speak to if they have concerns. Staff are attentive to changes in people's presentation, such as noticing smudged glasses or reduced appetite, and respond promptly and compassionately. During periods of ill-health, staff maintain close contact and visit people in hospital to offer reassurance and continuity. These actions contribute positively to people's sense of belonging, security and emotional comfort.
There is clear evidence that staff support people effectively through difficult emotional experiences. We saw an example where a person feels able to talk about the loss of a close relative following sensitive interaction and support of staff and the wider organisation. The service also arranges memorial plaques and planting in memory of relatives, creating shared opportunities.

The provider is working towards the Welsh active offer.

Care & Support — Good

People receive consistently good quality, person-centred care that reflects their individual needs and preferences. We saw an example where the provider chose not to offer someone a residency agreement as they considered how this would impact on residents currently living in the home. This demonstrated sound professional judgement and helps maintain the safety and stability of people already living at the home.

Personal plans are detailed, current, and promote independence, safety and well-being. Plans contain meaningful and personalised information, including family birthdays, preferred routines, hobbies, communication needs and people's own words, giving authenticity and ownership to the content. Care staff demonstrate an excellent understanding of what people can do for themselves and the provider is considering how this can be better captured in personal plans. The daily records reflect care staff's knowledge of people and demonstrate what people have done and achieved. Risk assessments are comprehensive, and behavioural support guidelines are well understood by staff. Incident forms are completed thoroughly and include lessons learned, evidencing a reflective and accountable culture.

People are supported to maintain good physical well-being through access to a wide range of healthcare services, with records and discussions with staff confirming that individuals see GPs, dentists, neurology and other specialists. When new or emerging needs are identified, timely referrals are made to health professionals. Staff demonstrate detailed knowledge of individuals' health conditions and the actions required to keep them well. This includes working closely with healthcare professionals to manage specific risks, such as concerns about blood clotting, ensuring appropriate treatment and clear communication to the team. During the inspection, responsive practice was evident, including a visit from the podiatrist, staff arranging an optician appointment after noticing a person's loose glasses, and a beneficial change following concerns about medication timing. Equipment, such as wheelchairs and pressure cushions, is well maintained, and individuals with specialist dietary requirements, receive safe and appropriate meals prepared and stored in line with guidance.

People are protected from harm. Medication systems are safe and well managed, and staff support individuals to be involved in their medication where appropriate. Staff demonstrate strong awareness of specific medication-related needs. Infection control arrangements are robust and support people's safety. Staff demonstrate a clear understanding of safeguarding responsibilities, and a recent safeguarding incident related to access to alcohol was appropriately reported, analysed and addressed. The Responsible Individual maintains regular contact with people and uses highly skilled communication, with consultation notes clearly showing that individuals' views are sought and acted upon.

Environment — Good

People live in an environment which meets their needs, supporting and encouraging wellbeing. The home feels warm and welcoming. Communal spaces are spacious and allow people to socialise, dine, and take part in shared activities. The kitchen is used primarily by staff, but residents are supported to participate in cooking or baking when they wish, promoting independence and life skills. People's bedrooms were observed to be warm and comfortable and decorated to individual choice. Bedrooms were equipped with adequate storage and personalised items reflecting people's interests and identity. This level of personalisation supports dignity, autonomy, and emotional wellbeing. People have access to private spaces to meet with family and professionals. The home is showing signs of wear but there is a plan in place to redecorate and replace some of the furniture.

Bathroom arrangements require attention. One resident with mobility issues is sharing an ensuite bathroom with another resident, compromising their dignity. Cleaning systems are in place but not effectively overseen, leading to a decline in standards. We saw clear plans for improvement. Outdoor areas are well maintained and include a lawned garden, patio space with seating and raised flower beds. Residents have recently planted daffodils, demonstrating opportunities for engagement with the environment and personal involvement in outdoor activity.

The home provides a safe physical environment that demonstrates proactive risk management whilst keeping people safe. Measures observed include radiator covers throughout the home to prevent burns, window restrictors fitted on first-floor windows to reduce risk of falls and fire extinguishers located throughout the building. The provider has recently installed new fire doors and door closers following an external fire inspection. The building is well maintained, and we saw evidence of relevant health and safety checks and audits carried being carried out. The service maintains detailed and accessible Health & Safety, Fire Safety, and Legionella policies and risk assessments. These reviews demonstrate a thorough understanding of potential risks and appropriate mitigation measures. Compliance certificates are well organised and easy to access. We saw evidence of regular safety checks including gas safety, electrical installation, PAT testing, fire equipment and emergency lighting. The service provides suitable equipment to meet peoples' needs.

Measures are in place to ensure visitors to the building cannot enter without having to be let in, and identity checked. All care records and personnel files are stored securely in a locked office.

Leadership & Management — Good

The provider has effective quality monitoring systems in place. The service uses audits to ensure care and support is provided as required. Feedback plays a significant role in shaping improvement within the service. The provider gathers views from people, families and staff in multiple ways, using this to make changes, address concerns and refine practice. While governance documents require some strengthening to formalise this learning, the culture of responsiveness, together with the Responsible Individual, (RI)'s active engagement, means feedback is regularly turned into action. This is ensuring the service continues to evolve and improve. Policies, records and safety measures are well established and kept up to date.

The service benefits from a highly visible and committed RI who demonstrates strong values-based leadership. People told us they know the RI well, and observations showed he engages naturally with individuals and staff, taking time to build meaningful relationships. The RI is frequently present in the home, participates in activities, and visits people in hospital during periods of ill health. This promotes a culture of trust, reassurance and emotional safety. Staff consistently described the RI as supportive, approachable and responsive.

The provider has strict selection and vetting processes which ensure staff are competent and trustworthy. The process is robust, incorporating competency testing, literacy and numeracy checks, and clear expectations around safe practice.

A comprehensive training matrix demonstrates strong engagement with learning and development, including specialist training to meet residents' needs.

There is evidence of a supportive staff culture. Supervision records show reflective discussions, competency checks and coverage of mandatory areas, and staff reported feeling encouraged and valued. Team meetings are detailed and incorporate training.

The workforce is generally stable, and the service does not use agency staff, supporting continuity of care. Staffing is described as flexible, with additional staff arranged to facilitate hospital appointments or outings. However, staffing pressures in recent months have reduced opportunities for some people to access the community as frequently as they wish.

Areas identified for improvement

Where we identify **Areas for Improvement** but we have not found outcomes for people to be at immediate or significant risk, we discuss these with the provider. We expect the provider to take action and we will follow this up at the next inspection.

Where we find outcomes for people **require significant improvement** and/or there is risk to people's well-being we identify areas for **Priority Action**. In these circumstances we issue a Priority Action Notice(s) to the Provider, and they must take immediate steps to make improvements. We will inspect again within six months to check improvements have been made and outcomes for people have improved.

CIW has no areas for improvement identified following this inspection.

CIW has not issued any Priority action notices following this inspection.

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